

STANDARD OPERATING PROCEDURE

SOP-CLN-002

Medical Officer Operations

Medivac.ai Platform

Clinical operating procedure governing how Medical Officers of the Royal Flying Doctor Service South Eastern Section use the Medivac.ai platform to support clinical decision-making, complex aeromedical retrievals, telehealth consultation and clinical governance.

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Document Control

This Standard Operating Procedure (SOP) is a controlled clinical document of the Royal Flying Doctor Service South Eastern Section (RFDS SE). It is issued in draft for consultation and must be read together with the RFDS SE Clinical Governance Framework and associated aeromedical operating procedures. Printed copies are uncontrolled; the authoritative version is the current release held in Medivac.ai.

Revision History

Version	Date	Author	Summary of change
Draft 1	July 2026	Perplexity Computer	Initial draft issued for clinical consultation and review.
v1.1	July 2026	Perplexity Computer	Updated: Referral PDF review requirement added to Section 3.

Approval

Role	Name	Signature	Date
Author	Perplexity Computer		July 2026
Reviewed by	Clinical Governance Committee		Pending
Approved by	Medical Director (Document Owner)		Pending

Related Documents

- SOP-CLN-001 — Flight Nurse Operations, Medivac.ai Platform.
- RFDS SE Clinical Governance Framework.
- RFDS SE Aeromedical Retrieval Clinical Practice Guidelines.
- Special Missions protocols — organ and tissue transport, ECMO, neonatal transport.
- After-Hours Medical Line operating protocol.

Definitions

Term	Definition
Medical Officer (MO)	A registered medical practitioner credentialed by RFDS SE to provide aeromedical clinical oversight, telehealth consultation and retrieval medicine.
Medivac.ai	The integrated aeromedical operations platform used by RFDS SE for dispatch, intake, telehealth, clinical documentation and governance.
Retrieval	The aeromedical transfer of a patient between locations under clinical care.
ORA	Operational Risk Assessment — the structured pre-mission risk review for a retrieval.
RSI	Rapid Sequence Induction — emergency airway management with induction and paralysis.
AHPRA	Australian Health Practitioner Regulation Agency.

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1 Purpose and Scope

1.1 Purpose

This procedure defines how RFDS South Eastern Section Medical Officers use the Medivac.ai platform to support safe, timely and clinically appropriate aeromedical care. It describes the Medical Officer's use of Medivac.ai to support clinical decision-making, to provide clinical authority over complex and high-acuity retrievals, to deliver telehealth consultations to referring clinicians, and to discharge clinical governance responsibilities.

Medivac.ai is a clinical support tool. It does not replace the professional judgement, clinical reasoning or accountability of the Medical Officer. Where this procedure describes platform functions — including any artificial-intelligence decision support — the Medical Officer remains responsible for all clinical decisions made in the course of a mission.

1.2 Scope

This SOP applies to all Medical Officers rostered to RFDS SE aeromedical operations, including those providing pre-mission clinical review, telehealth consultation, complex retrieval oversight, and after-hours medical line coverage. It applies to the use of Medivac.ai across the Dispatch & Intake, Telehealth Portal, After-Hours AI Med Line, Medical Equipment, Special Missions, Audit & Reports and Clinical Incident modules.

SCOPE EXCLUSION

This procedure does not govern nursing scope of practice (see SOP-CLN-001 — Flight Nurse Operations), aircraft operational airworthiness, or non-clinical dispatch logistics. Clinical practice guidelines for specific conditions remain authoritative for treatment decisions.

1.3 Objectives

- Ensure Medical Officers apply consistent, documented clinical processes when using Medivac.ai.
- Provide clear clinical authority and escalation pathways for complex and high-acuity retrievals.
- Standardise telehealth consultation, consent and documentation practice.
- Ensure AI-generated decision support is applied with appropriate clinical judgement and full clinical accountability.
- Support clinical governance, audit and continuous improvement.

2 Medical Officer Role in Aeromedical Operations

The Medical Officer is the senior clinical decision-maker for RFDS SE aeromedical retrievals. The role combines direct clinical oversight, consultative support to referring clinicians, and clinical governance leadership. Medivac.ai is the primary platform through which these responsibilities are exercised and documented.

2.1 Clinical authority on complex and high-acuity retrievals

The Medical Officer holds clinical authority over complex and high-acuity retrievals. This includes determining clinical suitability for transfer, defining the required level of clinical escort and equipment, setting the clinical management plan for the flight, and endorsing the mission's clinical risk. Where clinical opinion differs between the referring team and the retrieval crew, the Medical Officer arbitrates and records the final clinical decision in Medivac.ai.

2.2 Telehealth consultation for referring clinicians

The Medical Officer provides telehealth consultation to referring clinicians — including rural and remote medical, nursing and Aboriginal health practitioners — supporting assessment, stabilisation and disposition of patients prior to and during retrieval. Consultations are conducted and documented through the Medivac.ai Telehealth Portal.

2.3 After-hours medical line oversight

Outside standard hours, the Medical Officer provides oversight of the After-Hours Medical Line. This includes reviewing AI-generated clinical decision support presented through the After-Hours AI Med Line, responding to escalations, and retaining clinical responsibility for advice given under the medical line.

2.4 Clinical governance and audit

The Medical Officer participates in clinical governance through the review of clinical outcomes, incident analysis, and morbidity and mortality review. Medivac.ai provides the Audit & Reports and Clinical Incident modules used to identify, review and act on clinical risk and to support continuous improvement.

2.5 Responsibility summary

Function	Medical Officer responsibility	Medivac.ai module
Complex retrieval	Clinical authority, management plan, ORA endorsement	Dispatch & Intake / Special Missions
Telehealth	Assessment, advice, consent, documentation	Telehealth Portal
After-hours line	Oversight, escalation response, clinical accountability	After-Hours AI Med Line
Governance	Outcome review, incident review, M&M integration	Audit & Reports / Clinical Incident

3 Pre-Mission Clinical Review

Before a complex or high-acuity mission proceeds, the Medical Officer completes a structured pre-mission clinical review in Medivac.ai. The review confirms the clinical picture, the resources required, and the clinical risk, and ensures the correct protocols are applied for special missions.

3.1 Accessing mission details in Dispatch & Intake

Open the Dispatch & Intake module in Medivac.ai and select the assigned mission. Review the referral information, presenting complaint, observations, current management, and requested destination. Confirm the referring clinician's contact details and the clinical urgency recorded at intake.

3.2 Telehealth Portal — remote patient assessment

Where remote assessment is required, use the Telehealth Portal to conduct a remote patient assessment and to discuss the case directly with the referring clinician. Confirm the working diagnosis, response to treatment, anticipated clinical trajectory and any interventions required before or during transfer.

3.3 After-Hours AI Med Line — reviewing decision support

For after-hours cases, review any AI-generated clinical decision support presented through the After-Hours AI Med Line. Apply appropriate clinical judgement: the AI output is advisory only and must be corroborated against the clinical picture, the referring clinician's assessment and relevant clinical practice guidelines before any decision is made.

CLINICAL JUDGEMENT

AI-generated decision support informs but does not determine clinical decisions. The Medical Officer must independently verify the clinical reasoning and retains full clinical responsibility for the outcome.

3.4 Medical Equipment — confirming specialist equipment

Use the Medical Equipment module to confirm the specialist equipment required for complex cases. Verify availability, serviceability and configuration before the mission is authorised. Examples of specialist equipment include:

- ECMO (extracorporeal membrane oxygenation) circuit and consumables for critical cardiorespiratory support.
- Neonatal transport system — transport incubator, neonatal ventilator and monitoring.
- RSI kit — airway equipment, induction and paralytic agents, and difficult-airway adjuncts.
- Advanced haemodynamic monitoring, infusion pumps and blood products where indicated.

3.5 Special Missions module

For organ transport, ECMO retrievals and neonatal transfers, open the Special Missions module. Specific protocols apply to these mission types and must be confirmed as part of the pre-mission review. The Special Missions module presents the applicable protocol, the required team composition and the time-critical coordination steps.

3.6 Referral PDF Review

Referral PDF Review: Referring facilities may now attach a PDF referral document via the Hospital Referral Portal at mission intake. Medical Officers conducting pre-mission clinical review should access the mission record and open any attached referral PDF as part of the standard review process. The referral PDF provides the referring clinician's assessment, patient history, and specific transport or treatment instructions. Where a PDF referral is available, it takes precedence over verbal referral summaries

provided at intake.

4 Telehealth Consultation Workflow

Telehealth consultations follow a standard sequence to ensure consent, structured assessment and complete documentation. All steps are completed within Medivac.ai so that the consultation is captured in the mission record.

4.1 Launch the Telehealth Portal

Launch the Telehealth Portal from within Medivac.ai and open the relevant mission. Confirm the identity of the referring clinician and the patient, and check the connection quality before proceeding.

4.2 Obtain and document verbal consent

Obtain verbal consent from the patient or, where the patient lacks capacity or is a minor, from the parent or guardian. Explain the purpose of the telehealth consultation and record the consent — including who provided it and when — in the mission record.

4.3 Conduct a structured assessment

Conduct a structured clinical assessment using a recognised framework (for example primary survey / ABCDE and a focused history). Direct the referring clinician through examination and investigations as required, and reassess response to treatment.

4.4 Document findings, advice and decision

Document the clinical findings, the advice given, and the clinical decision in the mission record. Include the working diagnosis, management recommendations, and the rationale for the disposition and retrieval decision.

4.5 Confirm retrieval priority with the Dispatcher

Confirm the retrieval priority with the Dispatcher so that tasking and aircraft allocation reflect the clinical urgency. Ensure the agreed priority and any special requirements are recorded against the mission.

CONSENT

Verbal consent must be obtained and documented before a telehealth consultation proceeds. Where consent cannot be obtained and the patient requires urgent care, act in the patient's best interests and document the clinical justification.

5 Complex Retrieval Oversight

Complex retrievals require heightened clinical oversight, specific protocols and close coordination. The Medical Officer directs the clinical management plan and confirms that the crew, equipment and destination are appropriate to the clinical need.

5.1 Neonatal transport protocols

Neonatal transfers follow dedicated neonatal transport protocols. Confirm the transport incubator, thermoregulation, neonatal ventilation and monitoring configuration, and that the escorting team is credentialled for neonatal transport. Coordinate with the receiving neonatal unit and document the neonatal management plan in Medivac.ai.

5.2 Organ and tissue transport — time-critical coordination

Organ and tissue transport is time-critical. Coordinate closely with the donor and recipient teams and the transplant service, confirm cold-ischaemia constraints, and prioritise tasking accordingly. Use the Special Missions module to track the coordination steps and time-critical milestones.

5.3 Critically ill adult — RSI and ventilated patients

For critically ill adults, including patients requiring rapid sequence induction (RSI) or already ventilated, confirm the airway plan, ventilation strategy, sedation and haemodynamic support before transfer. Ensure the RSI kit is checked and that the escorting crew is competent to manage the airway in the transport environment.

5.4 Psychiatric and behavioural emergencies

For psychiatric and behavioural emergencies, complete a safety plan in Medivac.ai addressing risk of harm, sedation and restraint considerations, legal status under relevant mental health legislation, and the safety of the crew and aircraft. Document the safety plan and the agreed clinical and de-escalation approach.

SAFETY PLANNING

Behavioural emergencies present risk to the patient and the crew in a confined aircraft cabin. A documented safety plan in Medivac.ai is mandatory before a behavioural-emergency retrieval proceeds.

6 After-Hours AI Med Line

The After-Hours AI Med Line supports the Medical Officer outside standard hours by presenting AI-generated clinical decision support. The following principles govern its use.

6.1 Decision support only — clinical responsibility retained

The After-Hours AI Med Line provides clinical decision support only. It does not make clinical decisions and does not carry clinical responsibility. The Medical Officer retains full clinical responsibility for every decision and must apply independent clinical judgement to any AI-generated recommendation.

6.2 Logging of interactions

All After-Hours AI Med Line interactions are logged in Medivac.ai. The log includes the query, the AI-generated support presented, and the clinical decision recorded by the Medical Officer. These records form part of the clinical governance and audit trail.

6.3 Escalation

Escalate to the on-call Medical Officer whenever an AI recommendation is unclear, conflicts with the clinical picture, or the patient is high-acuity. Escalation is always appropriate where there is clinical uncertainty; the AI Med Line must never be used as a substitute for senior clinical review of a deteriorating or high-risk patient.

Trigger	Action
AI recommendation unclear or ambiguous	Escalate to on-call Medical Officer; do not act on ambiguous support.
AI output conflicts with clinical picture	Prioritise clinical assessment; escalate and document divergence.
High-acuity or deteriorating patient	Escalate immediately to on-call Medical Officer for senior review.
Any clinical uncertainty	Escalate — escalation is always an acceptable response.

7 Clinical Governance

Clinical governance ensures that RFDS SE aeromedical care is safe, effective and continuously improving. Medivac.ai provides the tools through which outcomes are reviewed, incidents are analysed, and lessons are integrated into practice.

7.1 Audit & Reports — monthly outcome review

Use the Audit & Reports module to conduct a monthly review of clinical outcomes. Review key clinical indicators, adherence to protocols, telehealth documentation completeness, and After-Hours AI Med Line usage. Record findings and actions arising from the review.

7.2 Incident review — Clinical Incident module

Report and review clinical incidents through the Clinical Incident module. Each incident is triaged, investigated proportionate to its severity, and closed with documented findings and improvement actions. Trends are monitored to identify systemic risk.

7.3 Morbidity and mortality review integration

Morbidity and mortality (M&M) review is integrated with the incident and audit processes. Relevant cases are referred to M&M review, and the outcomes — including learning points and system changes — are recorded and fed back into clinical practice and this procedure.

CONTINUOUS IMPROVEMENT

Findings from audit, incident review and M&M review must translate into concrete actions. Actions are tracked to completion in Medivac.ai and reviewed at the next monthly clinical governance meeting.

8 Documentation Standards

Complete, accurate and timely documentation is a professional and medico-legal requirement. The following standards apply to all Medical Officer clinical activity on the Medivac.ai platform.

8.1 All clinical interactions documented in the mission record

Every clinical interaction — assessment, advice, decision and escalation — must be documented in the mission record in Medivac.ai. Entries must be contemporaneous, attributable, legible and free of unexplained abbreviations, and must record the clinical reasoning behind key decisions.

8.2 Telehealth consultation notes — mandatory before next retrieval

Telehealth consultation notes must be completed before the Medical Officer commences the next retrieval. Notes must capture consent, the structured assessment, findings, advice given and the clinical decision. Incomplete telehealth documentation must be resolved before further clinical tasking is accepted.

8.3 Medical Officer endorsement of high-acuity mission ORA

For high-acuity missions, the Medical Officer must endorse the Operational Risk Assessment (ORA) in Medivac.ai. Endorsement confirms that the clinical risk has been assessed, the management plan and resources are appropriate, and the mission may proceed on clinical grounds.

Documentation item	Timing	Responsible
Mission clinical entries	Contemporaneous	Medical Officer
Telehealth consultation notes	Before next retrieval	Medical Officer
High-acuity ORA endorsement	Before mission authorised	Medical Officer
After-Hours AI Med Line record	At time of interaction	Medical Officer / system log

9 Compliance

Medical Officers must practise within the professional, regulatory and organisational requirements that govern aeromedical and telehealth care. The following compliance obligations apply.

9.1 AHPRA registration obligations

Medical Officers must hold and maintain current registration with the Australian Health Practitioner Regulation Agency (AHPRA) and practise within their scope of practice, recency of practice and continuing professional development obligations. Registration status must be current at all times while providing clinical services for RFDS SE.

9.2 Medical indemnity coverage for telehealth

Medical Officers must hold appropriate medical indemnity coverage that extends to telehealth consultation and aeromedical retrieval medicine. It is the Medical Officer's responsibility to confirm that their indemnity arrangements cover the scope of activity described in this procedure.

9.3 RFDS SE Clinical Governance Framework

All activity under this procedure must comply with the RFDS SE Clinical Governance Framework, including its requirements for credentialling, scope of practice, documentation, incident management and clinical audit. Where this procedure and the Framework differ, the Clinical Governance Framework prevails.

Obligation	Requirement	Verified by
AHPRA registration	Current, unconditional, within scope	Credentialling
Medical indemnity	Covers telehealth and retrieval medicine	Medical Officer
Governance framework	Full compliance; Framework prevails on conflict	Clinical Governance Committee

END OF PROCEDURE

This document is issued as Draft 2 (v1.1, July 2026) for clinical consultation. It is uncontrolled when printed. The authoritative version is held in Medivac.ai. Feedback should be directed to the Document Owner (Medical Director).